



Notes

May 5, 2026

Weight Management Research Readout

Invited Tzuyi Lee Zach Prong Brian Hill Matt Beidleman Lara Ortiz-Luis
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Attachments 📎 Weight Management Research Readout

Meeting records 📋 Transcript 🎙️ Recording

Summary

Cross-functional research session synthesized user needs for weight management programs via GLP-1 and body composition features.

Research Methodology and Prototypes

Qualitative interviews tested 3 distinct prototypes to identify user value propositions. Prototype 1 focusing on strength maintenance proved most compelling to participants.

Emerging Program Value Propositions

GLP-1 features and body composition tracking significantly outweighed other benefits. Participants prioritized convenience over manual logging while favoring provider oversight for medication safety.

Program Economics and Future Strategy

Affordability remains a primary driver for program adoption. The team

decided to prioritize 50 dollars monthly pricing to ensure competitiveness while maintaining accessibility.

Rate this Summary: [Helpful](#) or [Not Helpful](#)

Next steps

- ☐ [Sui] Assist Access: Help users having issues signing up or using the shared research document.
- ☐ [The group] Review Prototype: Examine the preliminary design prototype and add comments, thoughts, or sticky notes. Evaluate implications for product implementation.
- ☐ [The clinic team] Plan Implementation: Conduct a road mapping session to determine how to work backwards from the research findings.
- ☐ [Chris Lloyd] Coordinate Content: Discuss with fitness and content teams how to adopt content strategies based on research. Determine clinical program structure and required content.
- ☐ [Chris Lloyd] Determine Implementation: Analyze the overlap between the Bridge program and identified patient needs. Strategize how to implement necessary support features.

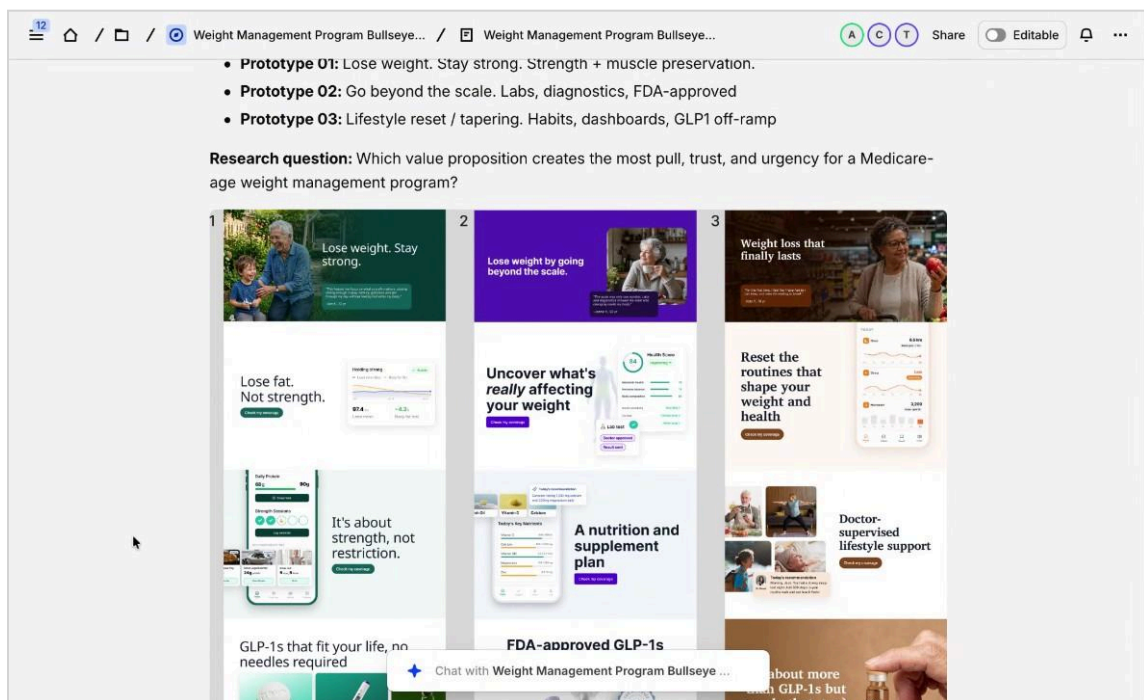
Details

Did the screenshots in this section make your notes [better](#) or [worse](#)?

- **Introduction to Weight Management Program Research:** The discussion began by welcoming participants to a cross-functional session intended to help digest user research findings regarding the future direction of the weight management program. The research focused on the potential for a GLP-1 companion weight management program and how to improve the current product experience, including nutrition programming and content, with potential immediate changes ([00:01:16](#)). They clarified that the current roadmap includes offering GLP-1s

directly to patients in Q3, and noted that Medicare will subsidize GLP-1s starting in July, which is expected to cause a significant increase in demand (00:03:15).

- **Research Methodology and Sample:** The research was conducted using a Bullseye customer research sprint, a method designed to identify a product opportunity that resonates with a specific segment of the market (00:04:34). The goal was to identify a group of customers who would be enthusiastic about the product's value propositions and willing to pay, rather than seeking broad appeal. The team conducted four qualitative interviews, selected from a large sample, noting that qualitative majority user needs can be identified with a small number of participants (00:06:13).
- **Prototypes Tested in Research:** The research tested three distinct prototypes: Prototype one focused on "staying strong," muscle preservation, dieting based on protein, strength exercises, and included form factors such as a pill or pen (00:07:24). Prototype two centered on "understanding your body," including lab testing, metabolic results, nutrient supplementation, and the safety and credibility of GLP-1s. Prototype three emphasized "lifestyle and lasting change," focusing on developing habits to taper off the drug, tracking lifestyle factors, physician guidance, and the option for a microdose of GLP-1s (00:08:52).



- **GLP-1 Factors Override Other Value Propositions:** The most significant finding was that GLP-1 factors strongly influenced people's decisions and tended to

override other value propositions. Features that resonated were the pill and pen options, name-brand mentions, FDA approval, and provider oversight for managing side effects (00:10:14). Participants disliked the idea of traditional injections and compound medication, and cost and side effects were identified as the two most important factors (00:11:30).

The screenshot shows a presentation slide titled "Results: Value prop hierarchy". The slide is part of a presentation titled "Weight Management Program Bullseye...". The slide content is as follows:

Results: Value prop hierarchy

Value Prop 1: GLP-1: Safety, Form Factor & Coverage

This was the first decision gate. People did not evaluate the rest of the program until they understood whether the medication felt safe, affordable, and tolerable.

What resonated

- Pill-first or pill/pen choice.
- FDA-approved medications.
- Clear coverage / affordability.
- Provider oversight and lab testing to avoid side effects.
- Avoiding or managing nausea, constipation, stomach issues, headaches, irritation, and unknown long-term effects.

What to avoid

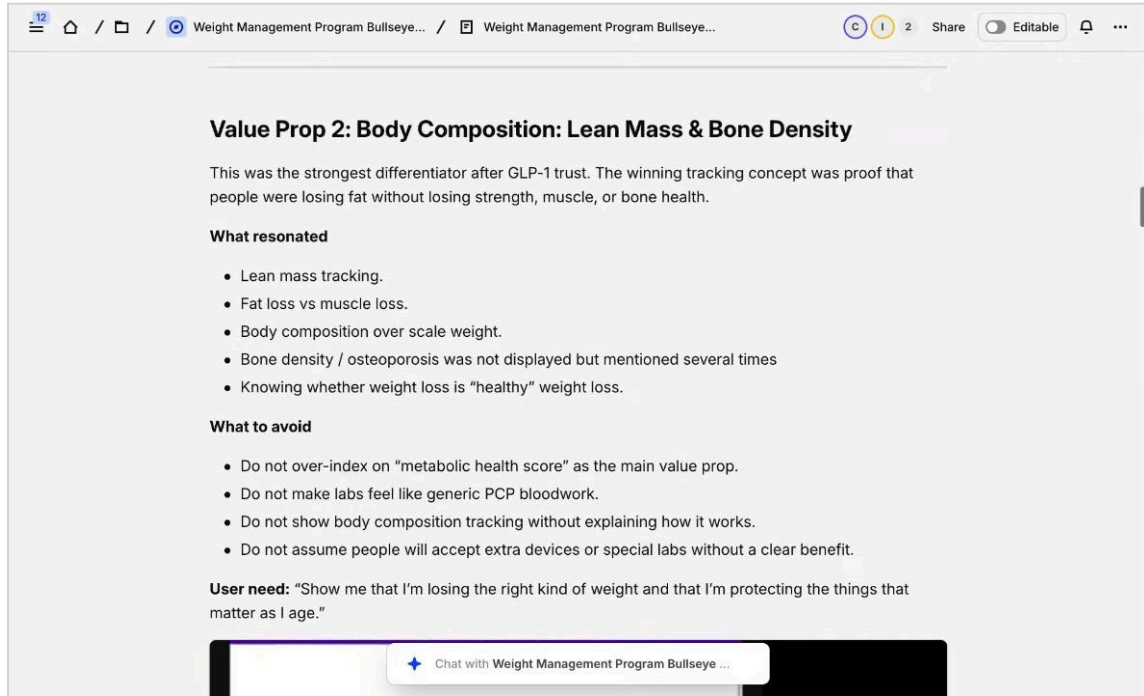
- Do not lead with compounded medication.
- Do not show a vial/needle as the hero treatment.
- Do not imply "GLP-1 access" without clarifying cost, coverage, safety, and oversight.
- Do not assume "FDA-approved" only creates trust; it also triggers cost/coverage questions.

User need

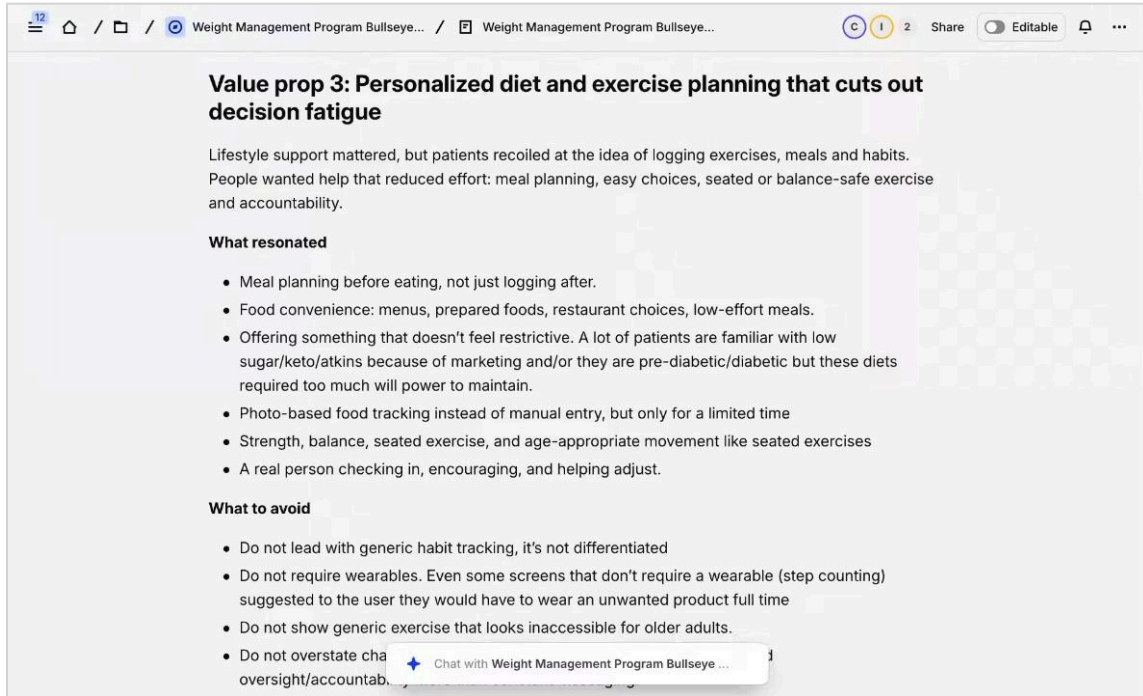
"Help me access a GLP-1 medication that is safe, affordable, and easy to take."

At the bottom of the slide, there is a chat bubble that says "Chat with Weight Management Program Bullseye ...".

- **Value Proposition on Body Composition Tracking:** The second most resonant value proposition was body composition tracking, which included tracking bone density in addition to lean body mass and fat loss compared to muscle mass (00:13:23). Tracking metabolic health did not resonate as much, as users felt they already receive that information from their primary care provider. The team noted that they need to determine how to support body composition tracking without making it feel like an overwhelming process for the user (00:14:37).

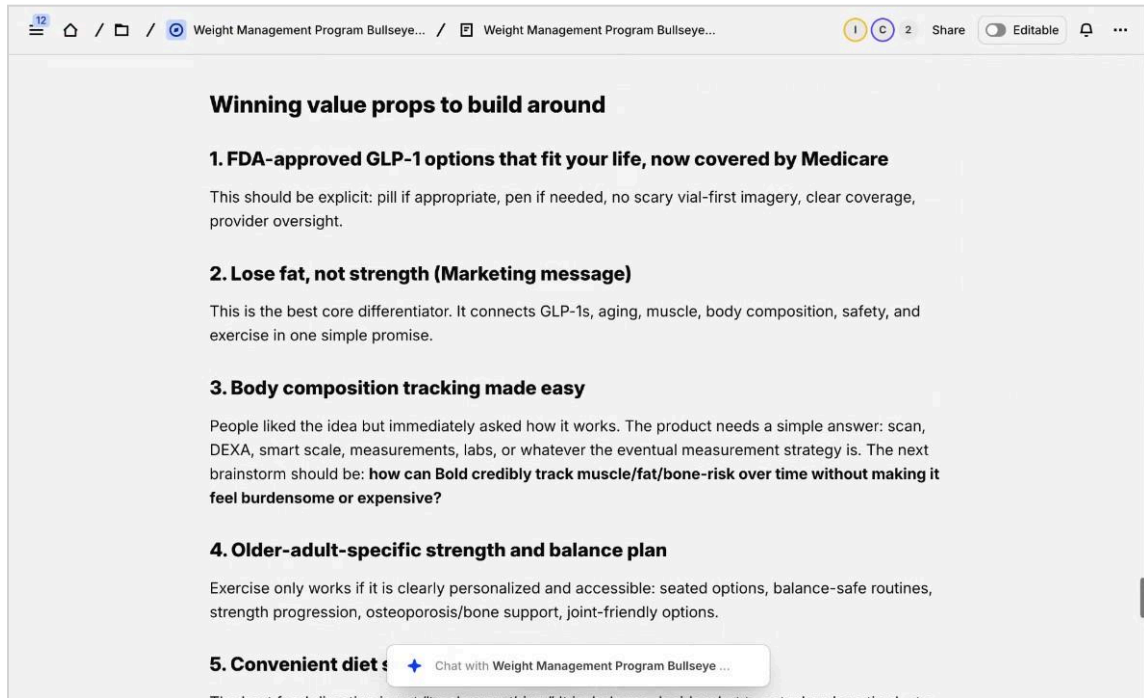


- **Personalized Diet and Exercise Planning:** The third key value proposition was a personalized diet and exercise planning feature that minimizes decision fatigue (00:15:42). While lifestyle support was important, it was less critical than the drug itself and the outcome tracking. Participants desired meal planning and exercise routines but rejected the idea of having to log another thing, especially manual entry (00:16:42) (00:19:54).

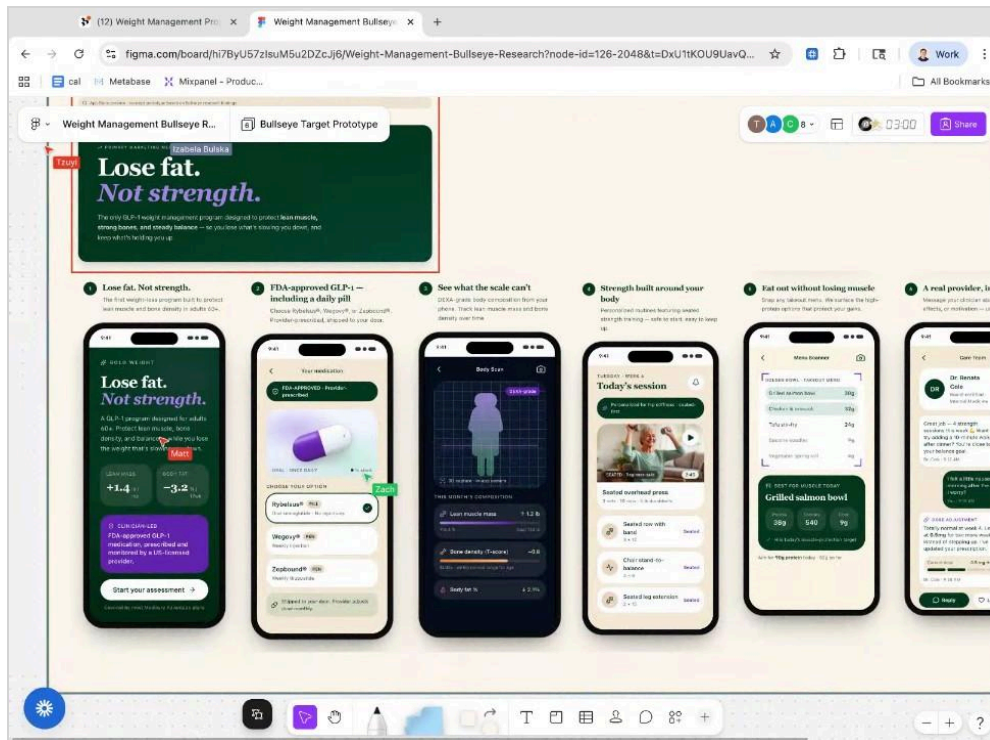


- **Preferences for Diet, Exercise, and Provider Interaction:** Patients expressed fatigue with restrictive diets and wanted convenient, non-restrictive guidance, especially for eating out or ordering delivery (00:16:42) (00:21:00). Helpful features included photobased tracking as an initial tool, and accessible exercise options, such as seated or chair exercises (00:17:45). Patients appreciated the idea of a physician checking in primarily for managing side effects, safety, and dosage, but not for frequent conversations or being told things they already know (00:18:50).
- **Prototype Findings and Key Insights:** Prototype one, focused on strength maintenance and the pill option, was the most compelling to participants. Prototype two was liked for its focus on body composition tracking, which needs to be integrated into the program, but metabolic health tracking was less relevant (00:21:00). Prototype three was the least liked due to the focus on tracking habits and less important doctor interactions, though some liked the idea of dosage adjustment (00:22:23).
- **Primary Value Propositions for Program Development:** Six primary value propositions emerged for the current and future weight management programs, including: offering FDA-approved GLP-1 options that fit their life, emphasizing the pill and Medicare coverage; using the marketing message "lose fat, not strength" for differentiation; integrating healthy body composition tracking features like

lean muscle mass and bone density (00:23:41). Other key points include offering accessible strength and balance exercises; providing convenient dietary guidance that avoids manual food logging; and ensuring a provider is in the loop for safety, side effect reduction, and accountability (00:24:50).



- **Discussion on Cost and Program Eligibility:** A discussion was held regarding the cost of the potential program, with the Bridge program offering prescriptions for \$50 per month, which is highly appealing compared to competitors costing hundreds of dollars (00:26:06). The people who were interviewed indicated a high likelihood of paying \$50 for the program, as high costs have historically prevented people from using GLP-1s (00:27:23). Not all participants had direct experience with GLP-1s, but most had been exposed to the negative side effects through stories (00:28:27).
- **Future Steps and Prototype Review:** The next steps include the clinic team conducting a road mapping session to determine how to integrate these value propositions and when to launch the GLP-1 program (00:31:12). Chris Lloyd requested that participants review and provide comments on a quickly developed prototype, which expresses the primary things patients wanted to see in the program (00:29:48). The team will also discuss program architecture and content with the fitness and content teams, and look at the overlap between the Bridge program offerings and patient needs (00:31:12).



- Inquiry on Menu and Recipe Sourcing:** Alicia Rios inquired about how the team plans to gather and provide menus or recipes for the convenience and diet value proposition (00:32:30). The plan is to brainstorm how to use technology, such as AI reading a photo of a menu, to provide convenient help before patients make eating decisions, rather than adding more logging tasks (00:33:46). This approach aligns with the need for practical tips, especially for older adults who may have different food preparation and dining habits .

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Transcript

May 5, 2026

Weight Management Research Readout - Transcript

00:00:09

Lara Ortiz-Luis: Hello.

Chris Lloyd: Hey,

Lara Ortiz-Luis: How's it going?

Chris Lloyd: I'm good. Is it chilly in the Bay

Lara Ortiz-Luis: Um,

Chris Lloyd: Area?

Lara Ortiz-Luis: it's just always cold in my house. Um, and I also got this ridiculous sweater that my cat fits inside, so it's like very silly.

Chris Lloyd: Nice.

Lara Ortiz-Luis: Yeah. Um, are you on your house boat?

Chris Lloyd: I am.

Lara Ortiz-Luis: How is

Chris Lloyd: It's cool. It's very cool. Uh I'm really Yeah,

Lara Ortiz-Luis: it?

Chris Lloyd: it feels nice. I am I don't know where we're going to live and what's happening, but right now it's like a little bit of luxury is helping

Lara Ortiz-Luis: Yeah, that's nice. Is all your stuff just in storage?

Chris Lloyd: Yeah,

Lara Ortiz-Luis: Wow.

Chris Lloyd: we brought some of it here and some of it's still in the house. We need to get rid of it, but basically everything was taken to storage last weekend.

Lara Ortiz-Luis: Wow. Nice.

Chris Lloyd: How are

Lara Ortiz-Luis: Um,

Chris Lloyd: you?

00:01:16

Lara Ortiz-Luis: I'm good. Yeah. Chugging along the, um, I feel like I know I messaged you a bunch of people's names from the

KK Dapaah: Hey everyone.

Lara Ortiz-Luis: longevity conference, but I feel like we should recruit them for content.

Chris Lloyd: Yeah, definitely.

Lara Ortiz-Luis: Hi, everyone.

Zach Prong: Hey.

Chris Lloyd: Welcome everybody.

Zach Prong: Hey.

Chris Lloyd: Folks a chance to get on. Welcome. Welcome.

Miranda Parry: Hey.

Chris Lloyd: Hey. All right, I might as well get started. I think we have everybody on. Um, so yeah, I I was going to do a smaller road mapping session and our interviews went until this morning and I just figured it might be better for people to digest some of the research that we did with uh users um around this kind of the future direction of our weight management program. And I've invited a crossf functional group of people because I feel like there's probably some implications uh in the short term that don't just have to do with us shipping a GLP-1 companion weight management program next quarter, but can be immediate changes that we make um in the short term for like how we're positioning the weight management program, how we're positioning um nutrition programming, and content um and product experience on the fitness product as well.

00:03:15

Chris Lloyd: Um so what I'm going to do is I'm going to share a little bit about the you know how we did the research, what we found. Please interrupt me with questions and then at the end I'll have a little bit of a chance for people to kind of like ask questions and respond to a prototype that I quickly spun up with some of the direction of what we heard um based on your kind of like initial reaction internally as well. Any questions before we get started?

Zach Prong: Just to be clear, we're not we're not offering GOP ones. We're just like this is like um we are supplementing the journey with GOP ones or of GOPs.

Chris Lloyd: Um we in our current road map we want to offer GLP1s directly to patients um in Q3.

Zach Prong: Got it. Okay, cool.

Chris Lloyd: Yeah.

Zach Prong: Thank you.

Chris Lloyd: Yeah. That's in the road map. Um, but I think right now we probably could be a GLP-1 companion program. There's probably people who are signing up for the clinic who are on GLP1s and I think they would find some of these um value props like resonant

00:04:34

Miranda Parry: Just to give just just like a very oneline context on that. Um from July, Medicare are subsidizing GLP1s for their beneficiaries. So, um they will be able to get JP ones for \$50 a month, which is incredibly um cheap. Um people are paying like 3 to 500 at the moment that that are on them. Um so we expect well there almost definitely will be a big uptick in demand but it's um that's the kind of relevance of that Q3 July um timeline.

Chris Lloyd: Yeah. Cool. Well, I'm going to jump in and maybe I'll record this, too, since I so I don't have to do it again. Um, I shared out this doc with folks. If anyone has issues signing up or using it, let maybe let Sui know and she can help you get in. Um, but this is great. Yeah. Um so just like a quick note on methods. Um we did something called a

bullseye customer research sprint which is designed at Google um by a group of user researchers and the purpose of it is to identify an opportunity to have a product that resonates with a wedge in the market.

00:06:13

Chris Lloyd: So the idea is not to test concepts that we think have broad appeal necessarily, but the idea is when you're starting a new product, you want to find a group of customers that are very enthusiastic about the value props of the the product that you are selling them and would be willing to pay. So what you do is you create three general hypotheses and you create variants on each of them that create three prototypes that are distinct. you do a little bit of discovery and kind of like getting to know the user. Um, and then they respond to each of the concepts. Um, and what you get is a kind of a hierarchy of uh value propositions that resonate or don't with your target customer. Um, we did four um interviews. We would have liked to do five. We had one no-show. Um, you might ask, how can you do user research with just a few interviews? There's actually a lot of documented evidence that uh, at least with qualitative research, you can get to the majority of identify the majority of user needs without speaking to many people.

00:07:24

Chris Lloyd: Of course, we'll want to test these things with larger like when we actually like have the final product, we'll be shipping features, doing AB tests, maybe we'll do some additional survey analysis, but you can actually get a lot of information really quickly with just speaking to a few people. These people were selected out of a huge sample of folks that um applied and responded to a 15 question survey um and kind of had like elements of people that we think represent the type of person that will sign up for this product as well as our you know history of who we've worked with within the clinic so far. We tested three prototypes. So one is um around kind of staying strong um and muscle preservation uh dieting around protein, increasing your protein um and uh uh strength exercises. And then along with that um having kind of like form factors uh

that were uh like you could use like a pill or pen or um like a traditional method. So like thinking about how the drug was administered. The second prototype was around um kind of like understanding your what's going on inside of your body.

00:08:52

Chris Lloyd: So lab testing, thinking about like metabolic results, um being able to um supplement your nutrients to make sure that you are staying like healthy and then focused on like the safety and the credibility of the uh of the GLP ones. And then the third one was more around like lifestyle and focusing on not staying on GLP once forever, but like developing the habits that would lead to lasting change where you could taper off of the drug, which included tracking like lifestyle factors, um having a physician in the loop who's providing guidance around lifestyle choices, and then um the option to have a micro dose or like lowest available dose of um GLP1s. These were all kind of from a larger brainstorm, but we felt like these were like the most interesting distinct variants on how we think about how you track your outcomes, what the patient would actually be kind of like doing, um what they would what's the lifestyle change that they would make and then how they relate to the value proposition of GLP1s. These all when you looked at them in more detail had more subheaders and stuff, but this is kind of the zoom out.

00:10:14

Chris Lloyd: The um kind of first um interesting outcome was that GLP1's in this uh experime or in this uh test like far outweighed any other value proposition and tended to like override people's decisions on other factors. people have really strong you know whether or not they personally have experience or know someone who has experienced side effects or if they have fear of needles um or other you know they had nausea when they tried GLP ones in the past um it really impacted their decision around which prototype they would use. The things that resonated were the pill um and having the option to do pen uh which is makes the injection a lot easier. Um any listing around like name brands, FDA approval. Um, sometimes when name brands were mentioned,

people had questions about the cost and then anytime there was a discussion around provider oversight um, specifically that was helpful as people thought that they might have side effects or other uh, symptoms that they'd want to manage with a physician. People really didn't like seeing things like around compound medication if they knew what that meant.

00:11:30

Chris Lloyd: um seeing things that suggested that it would be a kind of traditional injection. Um and then they had a lot of questions around cost. Um we didn't test coverage and cost because we believe that that is going to be a big part of our value prop regardless. Um but it will be important. It seemed to be like probably the most important factor along with side effects kind of competing for site um kind of like cost and side effects were the most important two factors. I'll share just a few clips. Um, we didn't have all of the users who were willing to record video, so some of them were quotes that we've captured below, but here's a few clips of people.

Chris Lloyd's Presentation: Last year I did GLP1 uh semiclude for

Chris Lloyd: Can you hear

Chris Lloyd's Presentation: like six weeks.

Chris Lloyd: that?

Chris Lloyd's Presentation: It was the worst experience of my life. Not only that, I only lost seven pounds. Cost me \$400 to lose seven pounds and I felt effects.

00:12:29

Chris Lloyd's Presentation: Yeah. Nauseous, constipation. Uh because I'm like, well, it was approved by the FDA and my insurance probably cover it. I'm like, woohoo, you know. Yeah, the pen. I don't think I'll ever do the pen again. That's a neat. The side effects is a problem. All right, that's number one. If it's okay, if I could do a pill, that'd be great. All right, pills easy. Pen second compounded with a vial like that. No way.

Definitely no. Definite no. I don't want to have to deal with drawing the medication into a needle. The needles that the needles are very how it it's so new. I mean, it just all of a sudden showed up. And I mean, in five years, are people going to have all these horrible, you know, illness? It was the shot. Now it's a pill.

00:13:23

Chris Lloyd's Presentation: I mean, much easier to do the pill for me because I'm definitely afraid of shots.

Chris Lloyd: So these are all available in this dovetail if you want to watch the videos um in uh full and then also share the transcript of Linda to do a recording. The second value prop that resonated the most with patients was around specifically tracking body composition. uh prototype one had something around like tracking lean body mass but also patients want to track bone density as well as an important element of body composition. So things like lean muscle mass uh tracking tracking fat loss over muscle mass um or compared to muscle mass thinking about the body composition compared to scale bone density um and then that was all framed within like making sure that it's healthy weight loss people don't necessarily just want to lose weight and a lot of GLP1 advertisements promise uh as their kind of brand promise is that you lose a certain percentage it's a percentage of your body weight. And so there's an unique opportunity for us to be differentiated here.

00:14:37

Chris Lloyd: Um some of the prototypes that showed tracking metabolic health didn't seem to resonate with people. They feel like they already get that with their uh primary care provider when they do lab testing. Um so that wasn't as helpful in the second prototype. Um they did have some questions about body composition and we'll have to deal with that internally as well. It's like how do you actually track that? Do I need to buy a device? Do I need to like go into and get lab results? So, we really need to figure out how we want to support tracking this composition and make sure that people don't feel

like it's an overwhelming process and it feels easier easy and that they can get value from it. Um, so that was a big uh component here and I'll play the videos that we heard from patients.

Chris Lloyd's Presentation: That's because I mean as you get older you have to worry about bone density and stuff but body composition is it stands out more than the rest of the other ones here.

00:15:42

Chris Lloyd's Presentation: Does I would help I would like to lose weight in the areas that I needed to lose weight. Right now my legs are very thin. All right. My thighs and my legs are very very thin. All right. I don't want to lose any weight there. That's good. Lose fat but not strength. That's that's good. Okay. Well, most most of my weight is from my stomach up to about here. All right. The lady is saying there that, you know, the scale tells you, you know, what how you're losing, but you really need to know how the formula works to figure it out. Yeah. Yeah. And that's probably where I've always gone wrong. That's why it always comes back.

Chris Lloyd: So the third value prop uh that stood out to people the most was around a personalized diet and exercise planning um feature that cuts phys uh uh decision fatigue.

00:16:42

Chris Lloyd: um lifestyle support mattered but not quite as much as the actual drug and then the outcome that you're tracking with the app. But people did find things like meal planning uh to be helpful um as well as like exercise routines that they could do. But patients did recoil from the idea of like having to log another thing. They did not like that idea at all. So, um, and a few people had kind of interesting, it surprised me, um, a lot of like how people talked about it, but you have this idea of like if you're 70 years old, you've spent so many decades like prepping meals for yourself, your children, your partner. Um, people want something. I think there's a lot of fatigue around like actually

like making food. Um, and so things like a, you know, if you're going out to eat or if you're ordering delivery, like what are some of the healthier choices that you could make that make it easy? A lot of patients have experience with like very restrictive diets like keto and stuff like that.

00:17:45

Chris Lloyd: And especially people who have are pre-diabetic or diabetic, they have kind of like low sugar diets which can be really tough uh to actually maintain. So, they have like a lot of like I think there's an opportunity for us to like reframe um a different way of thinking about diet that's more about this like building muscle, building strength, good for your bones and stuff like that and letting the GLP1 do the work of actually helping you with weight loss. Some of the features that thought that people thought might be helpful is photobased tracking um over manual entry, but people did not want to do that forever. It's more about like getting a sense of like what are the dietary like maybe it's something like you do at the beginning of the product to get things started so we have an understanding of what you're eating. Um people also wanted the exercise but um it's really important just like with our fitness product that we anchor it to how we showcase um exercise already that it's good for seniors, older adults, chair exercise options, personalized stuff like that.

00:18:50

Chris Lloyd: If people saw something that didn't fit them, they were likely to say, "Oh, yeah, this is not for me." Um, people did also like the idea of a physician checking in, but they didn't want to have a bunch of conversations with them. They felt like it might be helpful for accountability that they're making the changes in their lifestyle and then cared more about the physician checking in as it related to side effects, safety, dosage, and stuff like that. for the GLP1. People did not like uh things around like that that uh on one of the prototypes we had like a sleep and step tracker. They did not like that. They felt like it's something they can already get in other products and then also were concerned that it would require an additional device for them to uh buy or use or wear

all the time. Um they don't want wearables. Um they also yeah didn't like exercises that looked like they might not be able to do them um or inaccessible. Um and then the kind of chat with the physician felt like maybe not as useful to the patient.

00:19:54

Chris Lloyd: They just kind of felt like this would might be someone telling them stuff that they that they already know to do.

Chris Lloyd's Presentation: Well, I wouldn't want to have to put everything that I eat in there every day or every bit of exercise I do. Yeah. Um, that would be kind of very time consuming. So, it's actually in advance of the meal is more helpful to have guidance rather than after the fact. Is that right? All right. Yeah, like a diet or a a meal for something for you to manage weight. Um well, a a list of um like a diet um a specific diet listing, you know, for breakfast, you know, this food group for lunch, this food group, dinner, this food group, and then I could just, you know, follow that. Yeah. Um making it my own, not not getting frozen things. Um, and then an exercise program that maybe, you know, has you starting, my wife has basically stopped cooking.

00:21:00

Chris Lloyd's Presentation: She'll prepare once in a while, but we buy a lot of food that we take out, you know, from people that have prepared me. Dinner is usually 25% eating out, 75% bringing food in, prepared foods. But anything that requires me to be standing up for any length of time, that's a problem. I can't stand for more than maybe five or seven minutes.

Chris Lloyd: So those were the primary three. Um the result was essentially uh the primary three kind of insights that we got out of this. Um which meant that prototype one was the most compelling to people. Um it had the elements around maintaining strength, staying strong, losing fat, not strength. Um having a non-restrictive routine that was focused on um eating protein, um exercising, and then having a um especially this pill uh option as like an alternative to getting injections. Prototype 2 had some elements

uh that people liked. They really liked us elements of this, but it really turned out that they cared less about the metabolic health that was being tracked through labs and more about the body composition, bone density, uh, lean muscle mass, which was also covered in prototype one.

00:22:23

Chris Lloyd: But there's probably elements of how we presented this that might be compelling to add to prototype one. They also like the FDA approval and the name brands. Uh so as like a sign of safety, prototype 3 was the least liked one. Um people felt like tracking their habits was not as helpful and some of the interactions that were shown with a doctor were less important. Um when we got into it, some people liked the idea of the dosage um like micro doing um but more or less only really cared about the physician involvement to support and reduce um like side effects um and tapering off um or like adjusting the do dose over time. Um so that kind of led to oh have some more

Chris Lloyd's Presentation: This brown one is the third one we looked at. I guess the first one I like the the strength training that's mentioned the strongness that you can gain and track your strength and the um GLP one that fits your lifestyle all that kind of way. Yeah.

00:23:41

Chris Lloyd's Presentation: Yeah. This seems to be a lot of work. Okay. And also takes and 12 months. It's just kind of a repeat of the others. doesn't really give me anything have to I have an Apple Watch and I

Chris Lloyd: Okay, that's the the snippets that AI pulled for us. So, uh, forgive forgive us for the not great editing. Um, so I have a list of the kind of primary value props. And the reason some of these are very GLP-1 focused, but I actually think that we can start to adopt these in our current weight management program and um also the fitness product as it relates to some of the nutrition uh stuff that we're starting to do and weight management stuff. So the primary one is FDA approved GLP1 options that fit

your life. Um emphasizing the pill, but also that it's covered by Medicare. We may need to split out the coverage one as like a separate value prop. That's going to be extremely important to people, but we didn't test in this um in this round because we know that we are going to emphasize the the coverage.

00:24:50

Chris Lloyd: The second is lose fat, not strength. This is a marketing message of how we differentiate between other GLP-1 programs that promise specifically a percentage weight loss. We talk about losing the right kind of weight and not just like weight overall. On that uh kind of theme, we talk about body composition tracking. And we'll need to think about what is the right feature set for Bold to have that helps our members track a healthy body composition. Thinking about lean muscle mass and bone density. The fourth is going to be around strength and balance. um exercises that are weightbearing um but also offer seated options and accessible options. I think also from a clinical perspective, I'd love to start thinking about what what kinds of exercises actually are required to make sure that our members can maintain a good bone density um and preserve muscle to the best of their ability like a program that like aligns with that outcome. The fifth is around convenience and diet, not like blogging food. So getting helpful guidance that like helps people work in advance of when they eat to make the best possible decisions that supports their goals.

00:26:06

Chris Lloyd: And then finally, a provider in the loop to provide oversight and safety um specifically trying to reduce um side effects um and offering accountability. Um this is maybe needs to be pulled up a little bit higher potentially. Um we didn't test this like prototype or in the prototype as much but may need to be like emphasized a bit more. And that's that. I'm going to show a prototype of kind of like the target design. Um but maybe before I do that I would love to hear if there's any questions.

Zach Prong: This was awesome. Um, this was really really cool. Um, I had a I was curious about costs. So, I know we didn't do we didn't include costs for anything in this bullseye research. Um, but I'm kind of curious like how much they think they would pay for this product and like versus like how

Chris Lloyd: Yeah.

Zach Prong: much would we theoretically be actually um like making them pay out of

Chris Lloyd: Yeah. So,

Zach Prong: pocket?

Chris Lloyd: the program for Bridge is \$50 for the prescription or \$50 per month.

00:27:23

Chris Lloyd: I am not completely sure. I did ask people at the end if you had so I asked people like what their favorite prototype was if they wanted to bring in other elements of other prototypes to kind of like design their perfect program and then um basically how much would you pay for it and then I said if this was offered at \$50 would this be something that you'd pay from a from zero to 10 how likely would you be and the people that I had this conversation with said like nine um the anchoring comparison that they have is in the hundreds of dollars like \$400 700 range. And so if we are able to like work with Bridge and offer it at this rate, like I actually think it's going to be very appealing and cost has been the primary thing that's held a lot of patients back from from using it. Um the bridge program has other requirements of like who can get access to it, but um or people that are um eligible like \$50 is like a very appealing amount of money.

00:28:27

Chris Lloyd: And I think even if we had like a co-pay or other expenses involved with like seeing a physician, like we'd be well within the range of what someone um would think is reasonable for a product and would be excited to buy it. pricing is like notoriously a bad thing to ask people directly what they're willing to pay. Um, so I would also take that

with a with a grain of salt. It's kind of more in the moment. You kind of see if someone buys it or not is probably the best uh way to see if people are willing to pay.

Zach Prong: and got it. Cool. Thank you,

Chris Lloyd: Any other questions?

Zach Prong: Oh. Um, with this sample size, did they all have experience with GOP ones? Uh, I forgot if you mentioned that

Chris Lloyd: Uh, no, not all of them.

Zach Prong: earlier.

Chris Lloyd: I think three of the four had either I think tried them for like a short period of time and then one hadn't but had experience with someone had like everyone's heard about stories about someone doing GLP1 so people have been exposed to like some of the negative side effects like pretty universally is my expectation.

00:29:48

Chris Lloyd: Um,

Zach Prong: Got it.

Chris Lloyd: I think that's something that we'd need more of like maybe a larger survey to understand what percentage of people have actually tried it versus heard about it versus, you know, stuff like that would probably be better for a survey. Cool. Um, if people have time, I would love for people to look at this. So, this prototype I I spun up pretty quickly today. Um, let's give you guys a link and I would love it if people would add comments and thoughts to this. So, this has all of the kind of examples of all the primary value props that we discussed today. Um, so it has the primary marketing message. Um, some of the features that you'd expect to see based on what we heard. Um, and then, um, I did go ahead and add one for cost at the end since that was something I had forgotten to add. And I would love it if people would take uh 5 to 10 minutes to go through this, add comments and questions or stickies um thoughts of like how you might um position some of these things.

00:31:12

Chris Lloyd: If there's things that are coming up for other parts of the product or questions about how to implement, I would love from everyone's perspective to get like a quick voice over uh of what people think. Um, this is not going to hurt my feelings if you think it sucks or you have like negative comments because I just basically designed this in lovable in like 10 minutes. Um, but it's an expression of what we heard from patients as like the primary things that they wanted to see in a program. Cool. Great. Well, I would love it if people would take a few minutes to respond. I'll I'll kind of take a peek. Um, are there any other questions or thoughts that came out of this that people want to share? Um, I think clinic next step is that the clinic team will do a little bit of a road mapping session around how we want to work backwards from this. I think there are things that we can change in the product right now that are anchored to some of these weight management value propositions and then we'll think about when we want to launch this GLP1 program.

00:32:30

Chris Lloyd: I would love to also talk with the fitness and content side of the house as well around how we can adopt some of these um more content uh like create this what do

Brian Hill: All

Chris Lloyd: we think this like program is going to look like from a clinical perspective the content that we need and then I also think that there's you know we'll have to talk about roadmapping and sequencing but when do we want to offer

Brian Hill: right.

Chris Lloyd: people the opportunity to focus on weight management as a primary motivation of joining bold and if we do or we want that to be a separate product. I think there's some product architecture questions that we might have there. And then um I think Miranda and KK had to drop, but that's the other piece is looking at the overlap of the bridge program and some of the things that we know that patients want and figure out how to how we'll implement that um and support those those needs. Alicia

Alicia Rios: Yeah, sorry. Um, have we given it any thought as to how we are gathering these um menus or yeah, how are we providing that service or where are we gathering these recipes from?

00:33:46

Alicia Rios: Is there any thought around that?

Chris Lloyd: Not yet.

Alicia Rios: Okay.

Chris Lloyd: Um, you know, something that comes to you, it's very easy to take a photo of uh a menu and have AI read it and give you recommendations. Uh, I think I just saw a Google Pixel commercial where someone does that because they are at a fancy French restaurant and don't understand what the what's on the menu. Um, so there's definitely like ways to solve for that. I think the underlying user need we want to address is like creating a being helpful in a convenient way to the patient and not giving them more tasks to do to log food. we want to help them before they are making decisions around what they're eating rather than asking them to do more work after. Um, and so there's probably some brainstorming to do around like how we can do that uh most effectively.

Alicia Rios: Okay,

Chris Lloyd: Yeah,

Alicia Rios: thank you.

Chris Lloyd: it's kind of similar to I think what the people are asking in the nutrition class, which is like what are practical tips I could use when I'm eating out and stuff like that because people have I think a little bit maybe I don't know different uh lifestyle than people at you know a younger demographic around like how they are preparing for food. Um that's really anchored to

Alicia Rios: Yeah.

Chris Lloyd: convenience.

Alicia Rios: Thank you. Yeah. And considering that probably some of them are coming also maybe from communities where they don't have um they don't prep their meals so restricted.

Chris Lloyd: Yeah. Yeah.

Alicia Rios: Okay. Thank you.

Chris Lloyd: All right, thanks everybody for joining. Um, if you can drop comments,

Brian Hill: Awesome

Chris Lloyd: I would love for people to to add them. I appreciate that. Bye guys.

Brian Hill: work. This is super cool.

Zach Prong: Thank you.

Brian Hill: Thanks everybody.

Chris Lloyd: Okay,

Zach Prong: Bye.

Chris Lloyd: bye bye.

Transcription ended after 00:35:42

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